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Diagnostic stickiness and shifting relevance: Reassessing autism spectrum disorder (ASD) and/or attention-deficit hyperactivity disorder (ADHD) diagnoses. A qualitative interview study

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ABSTRACT

The rising rate of diagnosed autism spectrum disorder (ASD) and attention-deficit/hyperactivity disorder (ADHD) has led to more individuals near the diagnostic threshold living with neurodevelopmental diagnoses. While often assigned in childhood to initiate support in medical, school and family contexts, these diagnoses may persist across time and settings, shaping adult life in ways that are unaligned with current functioning.

This study explores the perspectives of individuals who actively sought to have ASD and/or ADHD diagnoses reassessed. Within a pilot project in Skåne, Sweden, fourteen individuals aged 17–30 years, originally diagnosed at ages 4–18, participated in semi-structured interviews. Motivations for seeking reassessment, examined using reflexive thematic analysis, clustered around four themes: having capacity, being no different, demanding fairness, and removing barriers. While some initially benefited from resources linked to diagnosis, many described how it had become restrictive—limiting career options, influencing health-care encounters, and shaping identity. Diagnoses were experienced as "sticky": tied to childhood contexts yet continuing to circulate into adult institutions.

Although not representative of all individuals with ASD/ADHD, this group constitutes a critical case, highlighting reassessment as a possible response to the temporal durability and institutional reach of psychiatric labels. The very fact that individuals request, and institutions require, a formal act of removal underscores the enduring power of such classifications as both resources and constraints. The study contributes to debates on overdiagnosis by highlighting the temporal and institutional dimensions of diagnostic expansion, and by examining how proportionality between benefits and burdens may shift over time.

1. Introduction

Over the past two decades, diagnoses of autism spectrum disorder (ASD) and attention-deficit/hyperactivity disorder (ADHD) have increased markedly in Sweden and internationally (Polanczyk et al., 2007; Lundström et al., 2015; Arvidsson et al., 2018; Rydell et al., 2018; Hansen et al., 2021; Jablonska et al., 2024). This expansion has been widely discussed in relation to shifting diagnostic thresholds, heightened awareness among professionals and the public, institutional incentives, and changing expectations about behaviour and

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performance in school and work (Garcia-Argibay et al., 2024; Conrad and Bergey, 2014; Duvall et al., 2025). While such developments are often analysed through the lens of medicalization – describing the expansion of medical categories into new domains of life – they have increasingly also been discussed in terms of overdiagnosis.

In epidemiological research, overdiagnosis is commonly defined as the identification of conditions that would not have caused symptoms or harm, or situations in which diagnostic expansion does not yield proportional benefit to patients (Brodersen et al., 2018; Moynihan et al., 2012). This literature has been most extensively developed in relation to cancer screening and other somatic conditions. However, the concept has also informed broader debates about diagnostic expansion, lowered thresholds, and the balance between benefit and harm in mental health (van Dijk et al., 2016; Moynihan et al., 2012). In these discussions, overdiagnosis does not necessarily imply diagnostic error at the time of assessment. Rather, it draws attention to questions of proportionality and to situations in which the overall effects of diagnosis, including treatment, labelling, and institutional consequences, may be limited, ambivalent, or detrimental over time.

Much of the literature on overdiagnosis has focused on diagnostic expansion at the point of classification, including changing thresholds, revised criteria, screening practices, and rising prevalence rates (Welch et al., 2011; Brodersen et al., 2018; Glasziou et al., 2020). Less attention has been directed towards what happens over time, once a diagnosis has been recorded and begins to circulate across institutional contexts. Diagnoses, particularly psychiatric ones assigned in childhood, often endure well beyond the clinical contexts in which they were first established. They travel into educational systems, welfare bureaucracies, employment sectors, and licensing authorities, acquiring new meanings and consequences along the way (Jutel, 2009, 2024). In this sense, overdiagnosis may not only concern initial classification, but also the temporal durability of diagnostic labels beyond their clinical and functional relevance.

This study explores a group that makes this temporal dimension visible: young adults who actively sought reassessment of previously assigned ASD and/or ADHD diagnoses. Within a pilot project in southern Sweden, individuals who had received neurodevelopmental diagnoses in childhood requested renewed evaluation, often asking whether the diagnosis still applied – and in many cases, whether it could be formally removed. These individuals are not statistically representative of all people with ASD or ADHD. Rather, they constitute a critical case that illuminates how diagnostic categories persist, travel, and are retrospectively contested.

By examining participants' motivations for seeking reassessment, we aim to contribute to social science debates on overdiagnosis and diagnostic expansion. Instead of asking whether particular diagnoses were "correct" or "incorrect," we analyse how individuals negotiate the continued presence of a classification that may no longer align with their current functioning, identity, or life aspirations. Their reassessment requests make diagnostic thresholds visible as temporal and institutional boundaries, rather than as one-time clinical decisions. In doing so, we shift attention from the initial act of diagnosis to the afterlives of diagnostic categories, and to the institutional and personal work required to undo them.

The marked increase in ASD and ADHD diagnoses illustrates what Conrad and Potter (2000, 2014) describe as the diffusion of medical categories, where diagnostic boundaries extend to new populations and life stages. In parallel, there is growing unease within the scientific community as well as among clinicians regarding the mismatch between observed diagnostic rates and epidemiological expectations. As diagnostic nets widen, more people whose functioning is close to the "typical" range come to live with neurodevelopmental diagnoses. This trend resonates with sociological debates on medicalization, where diagnostic categories expand to include increasing varieties of human experience, and with discussions of overdiagnosis, which draw attention to questions of proportionality and potential unintended consequences when diagnostic expansion does not clearly translate into sustained benefit (van Dijk et al., 2016; Busfield, 2017). Together, these perspectives highlight how expanding diagnostic categories reshape boundaries around what counts as "normal" or in need of care. Our study contributes by examining how such processes may later be reconsidered through renewed assessment of existing diagnoses.

Cases at the edge of diagnostic thresholds are analytically important, not because they settle where the diagnostic boundary should be drawn, but because they reveal a paradox at the heart of diagnostic expansion. The same label that can unlock needed support in childhood may later operate as a barrier to full participation in adult life, raising questions about the proportionality between benefits and burdens over time, as diagnoses are taken up by institutions (e.g., education, military, police recruitment, licensing authorities, health-care triage) as sorting devices with durable consequences (Sims et al., 2021). Diagnostic categories become *social resources* as well as *social constraints*, shaping access and exclusion across time and institutional domains (Jutel, 2011, 2024). In line with Jutel's (2009, 2011, 2024) work in the sociology of diagnosis, diagnostic categories can be viewed as social as much as medical objects that organise meaning, authority, and responsibility across institutions. Diagnoses are not only clinical judgments but social classifications that carry moral weight and institutional consequences over time. Beyond institutional effects, psychiatric diagnoses also carry social and emotional consequences. As O'Connor et al. (2018) show, diagnostic labels can both undermine and enhance young people's self-concept, producing experiences of alienation and stigma but also self-understanding and belonging. These ambivalent effects show that diagnoses function not only as clinical categories but as social identities negotiated in everyday life.

Our study examines individuals who, as young adults, actively re-engage with and challenge earlier diagnostic decisions, addressing a research gap on adult retrospective reflections on childhood diagnoses. We aimed to explore who seeks to have a diagnosis removed, and why. How do they describe their experiences of receiving, living with, and seeking to undo a diagnostic label?

2. Methods

Between 2023 and 2025, a pilot project was conducted within child and adolescent psychiatry (CAP) in Skåne, Sweden, in collaboration between the University of Gothenburg, Lund University, and the regional healthcare provider Region Skåne. The project aimed to study the need for and structure of a routine for reassessments of ASD and ADHD. In this descriptive inductive qualitative study interviews with participants in the pilot were analysed by reflexive thematic analysis. In a constructionist approach we aimed to recognize the subjective perspectives of the study participants (Braun and Clarke, 2006, 2019).

2.1. Setting and participants

Region Skåne, with a 1.4 million population, is situated in the southern part of Sweden, which in 2022 had an estimated prevalence of ASD/ADHD of about 0.8/2.6 percent among individuals aged 0-19 years (Jablonska et al., 2024). At the time of this study, the region annually received about sixty requests from residents who turned to healthcare and asked about the possibility of having ASD and/or ADHD diagnoses removed. Reassessments of ASD/ADHD were not part of standard healthcare, wherefor our project aimed to assess the need for such a routine.

Individuals contacting Region Skåne's child or adult psychiatric units requesting reassessment of ASD and/or ADHD were referred to the project. Inclusion criteria were age >16, residency in Skåne, and a registered ADHD and/or ASD diagnosis. Individuals who had ADHD medication within the previous 12 months were excluded. Seventy-two were included, aged 16-57 years, whereof 80 percent were aged 16-24. Over half had a prior diagnosis of ADHD. Eleven were recommended to

retain their diagnosis. An additional three were awaiting further assessment. The subsample of participants who were qualitatively interviewed, consisting of the first 14 out of the 72, were 17-30 years old, whereof ten males and four females. All except one were in employment or studying. Five had ADHD, four had ASD and five wished to reassess both diagnoses, received at 4-18 years of age by public healthcare providers. One participant had had diagnosis for less than one year, seven for five to nine years, and six for ten to fourteen years. Ten had taken diagnosis-related medication.

In clinical evaluations, current and childhood symptomatology among all 72 were assessed through structured interviews, followed by case conferences involving experienced clinicians who jointly assessed whether the individuals currently and at least for the last 1-2 years still met diagnostic criteria for ASD and/or ADHD. Eleven of the 72 were recommended to retain their diagnosis and an additional three were awaiting further assessment. Out of the 14 who were qualitatively interviewed in this specific study, twelve were classified as not to meet criteria for the original diagnosis and as a result had it removed, meaning that it was noted in their medical records that they did not have the diagnosis/-es. One had their ADHD diagnosis first removed, and six months later, ASD. One participant was assessed to have a low level of functioning and significant difficulties, such that a diagnosis and continued contact with habilitation services was deemed relevant.

2.2. Data generation

Interviews, performed in Swedish, were conducted June-August 2023 after participants had begun to take part in procedures related to the reassessments of their diagnoses but before reassessments were finalized. A semi-structured interview guide was used, including open-ended questions related to the reasons behind seeking reassessment, meanings of the diagnosis in the participants' lives, experiences related to diagnostic assessments and reflections about the future. A pilot interview, also included in the analysis, was performed by RL and LE together, both experienced in qualitative interviewing. The pilot resulted in minor changes to the guide, including asking more specifically about circumstances where the diagnosis may turn up or play a role in transitions between institutions, such as the shift from elementary school to high school. The following interviews were performed by RL alone. Recruitment and interviewing continued until the material was judged to provide sufficient depth, variation, and information power in relation to the study aim. At this point, the interviews had captured recurring themes across participants' accounts while still allowing for differences in how reassessment was motivated, experienced, and anticipated. The final sample consisted of 14 interviews. Follow-up questions were asked based on participants' responses, allowing the interviewer to pursue issues of particular relevance and gain depth in each account. Follow-up questions were asked based on the participants' responses and with the aim of following the participants to gain depth. Seven interviews took place in physical meetings at hospital premises and seven remotely, by video call. Interviews lasting 28-102 min, were audio-recorded and transcribed.

2.3. Analysis

Data analysis was performed collaboratively by co-authors RL, KN, AL, and LE. The analytic process combined individual work with joint meetings, including full day analytical sessions and the sharing of notes. We allowed substantial time for reflection on the interview material, making use of our different backgrounds in social science, psychology, theory of science, and medical science. These different disciplinary backgrounds, theoretical orientations, and roles generated productive discussions, enriched our interpretations, and supported reflexivity throughout the analysis, as emerging interpretations, assumptions, and uncertainties were continuously examined among the co-authors.

Initially, to become familiar with the data, we listened to recordings and read and re-read the transcripts. To generate the initial codes, we began by individually reading the first ten pages of three interviews, taking notes on preliminary thoughts and ideas, and highlighted data extracts relevant to the aim. We then discussed the highlighted excerpts and drafted a first version of the coding scheme through joint reflection. Text excerpts were coded inductively and close to participants' own words. NVivo [QSR International Pty Ltd. (2021) NVivo, Version 14] was used to support the analysis. The preliminary coding was expanded and adjusted through analysis of the remaining interviews. Throughout this process, we continued to review interviews individually, and to meet regularly to reflect on content, coding and emerging interpretations.

When all interviews had been coded, analytically significant features in relation to the aim were collected into themes and organized into clusters based on meaning. Relevant data excerpts were then collated to develop a concise and coherent account of the data across themes.

2.4. Ethical considerations

Study participants were recruited by KN who also conducted the initial clinical assessments. Participation was voluntary and based on informed consent. Interviewers were not involved in the reassessment process, and the clinical team involved in assessments did not gain insight into the data until after the participants had been assessed. To ensure anonymity, data was presented without reference to age, gender, or workplace. The research protocol (reg. number 2022-06213-01) was approved by the Swedish Ethical Review Authority.

3. Results

Our analysis rendered four main themes capturing different important elements related to the participants' rationale to seek reassessment of ASD and/or ADHD. The themes, illustrated as circles at the centre of Fig. 1, related to who the participants perceived themselves to be (*being no different* and *having capacity*) and what they aspired to achieve when asking for reassessment (*demanding fairness* and *removing barriers*). Content relating to the themes are presented in Fig. 1 to provide a brief overview of results, which are expanded upon, linked to the themes and exemplified with data excerpts in the text below.

Participants gave rich descriptions of how being diagnosed affected them in the past, present, and future. Content is presented chronologically with descriptions of receiving the original diagnosis, its effects, followed by a presentation of the themes. Thereby, we hope to provide an easy-to-follow description of the stories, gradually building towards the participants' rationale to seek reassessment of ASD/ADHD.

3.1. Being diagnosed: a confusing and unfair process with limited influence

Participants described the original diagnosis acquired in childhood as a process without control, dominated by the adult world. The process leading to diagnosis had been initiated by parents, sometimes supported by teachers, other school personnel or social services. Participants described how adults had acted due to being alarmed about what they had perceived to be strange or irregular behavior at home, in school or elsewhere, of "something not being right" (Interview 7).

Participants reported having displayed characteristics and behaviors at the time which they thought explained being diagnosed. These included having been too lively and unable to sleep, too lazy and unfocused, having had difficulties with language, food or getting started or having been unable to adjust to changed circumstances, being hyperactive and angry.

> " I did everything to just do the opposite. (…) I was very, like, loud and disturbed." (Interview 5)

> "I was awful as a kid. I was aggressive." (Interview 12)

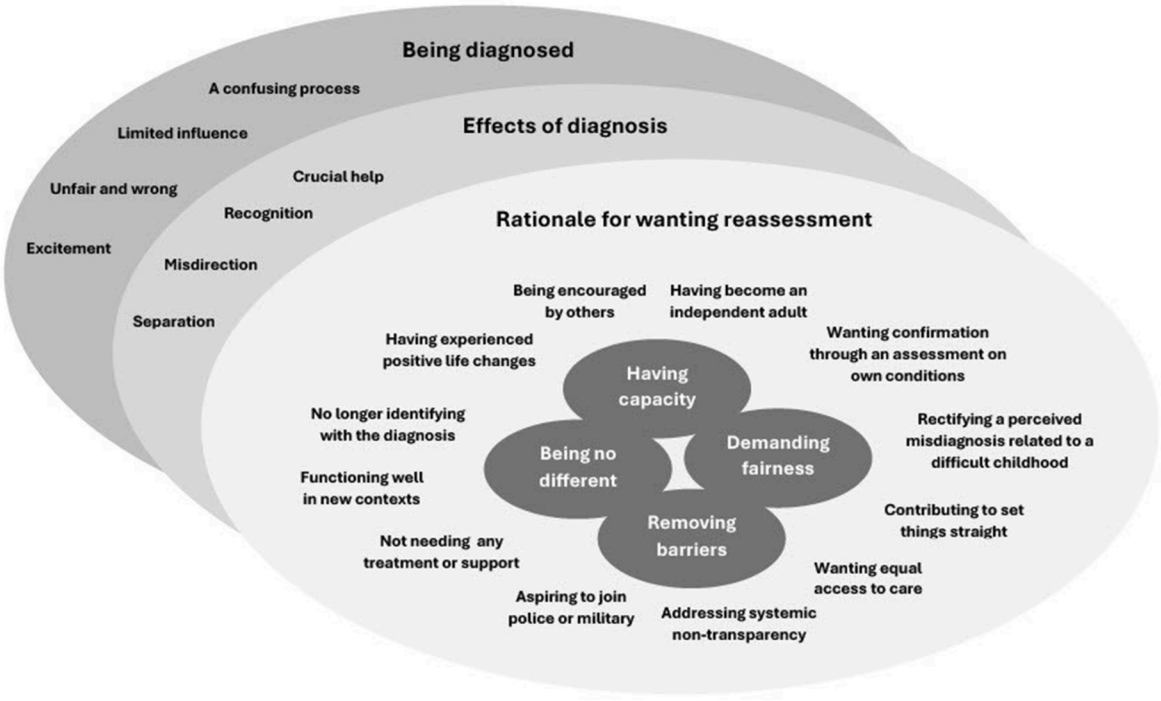


Fig. 1. Thematic map showing four interrelated main themes (Having capacity, Being no different, Demanding fairness and Removing barriers) regarding descriptions of rationale to seek reassessment of ASD and/or ADHD diagnosis and content relating to the themes (free text surrounding the themes).

”I was very rowdy och (…) didn’t listen much.” (Interview 11)

Participants described how they had had low motivation in school, social difficulties, constantly gotten into fights, and not having wanted or been able to interact.

” In eighth grade, when I really didn't go to school at all, I didn't have any friends to talk to at all. (…) I was at home gaming.” (Interview 2)

”I wasn’t the one who played with the others.” (Interview 14)

Some participants’ views on these characteristics and behaviors were that was just who they were at the time, not like everyone else and just different, slow learners, withdrawn or lonely, and that these behaviors had been interpreted as signs of diagnosis. Participants expressed having experienced unfairness in the process of being diagnosed as their reactions to unsafe, stressful or traumatic living conditions had been misinterpreted as diagnostic symptoms, mentioning parental addiction, bullying, deaths in the family, and exposure to abuse. Participants explained that these difficulties in life had made them lose motivation and concentration.

“There is so much in my personal life that has affected me at school, which has perhaps led to me receiving this diagnosis.” (Interview 7)

”He (the father) abused my mother so much that he ended up in prison when I was little. (…)I had to sit there with a tape recorder to my ear so that they could record conversations with him, because you never knew what he was capable of. So, that was very scary. (…) There was a lot happening there that made my behavior escalate.” (Interview 4)

Several participants described their parents as having been persistently driving the process towards a diagnosis, in a targeted way. They said they had felt exposed to procedures where adults around them were “fishing for faults” (Interview 14), and teaming up against them.

”Dad said: “Hey? Wanna go somewhere?”, without knowing where we were going. But just to get me out of the house. And then we went to CAP in [city]. And then I had to answer questions. (…) And then, just out of nowhere, it was: “Well, you have this diagnosis”.” (Interview 12)

”Mom said that I would get help by going to school and then a teacher tells me not to go to class, that I was going somewhere else in her car.” (Interview 13)

The neuropsychiatric assessments were described by some as blurry and based on information from others.

”They diagnosed me based on what the teachers and my parents said.” (Interview 5)

Participants described having felt confused about the meaning of meetings and tests and how they had willingly or with little resistance tagged along filling out papers and doing tests.

” Most of the time I was quite bored. (…) Like, I was just excited to go to the playroom.” (Interview 6)

”A really strange place where I met an even stranger person who said he was a doctor and was going to help me by giving me a math test.” (Interview 13)

Participants described different levels of confidence towards those leading the assessments, where some had felt like they had been based on standard checklists. They had been handed a diagnosis, like ‘here you go’, without sensing that the procedure was focusing on them as individuals and without looking for alternative explanations for their behaviors. Some participants said that they found it strange and questioned if it was reasonable to, for example, diagnose a child who did not have a safe home environment. Some described being angry and upset about processes they had felt forced to go through.

”He's the one who gave me this diagnosis, he's the one I see as the biggest bad guy in this. Him and the Swedish system. (…) I remember refusing to go there and how I hated him with my whole being.” (Interview 12)

Some participants described having completely trusted the professionals and the procedures within psychiatry. Others described that

no matter what they said or did was interpreted as typical signs of diagnosis.

> ”I felt a bit manipulated (…) Like, I told her: “Yes, but I’m very sensitive, I get easily offended by others”. And they just said: “Yes, but that’s very common among autistic women”, and then it was like this: “Yes, but I’m outgoing and I don’t have a problem with, like, social interactions”, (…) and they just said: “Yes, but that’s also very common in autism in women”, and no matter what I said, they just said: “Yes, but that’s … it’s not in the book, but it’s common among autistic women.” (Interview 8)

Participants commented on how adults around them had motivated the need for and benefits of being diagnosed, mainly to gain access to resources in school, and to receive financial aid.

> ”They said that I was diagnosed with high-functioning autism, uh, mainly to be able to get help at school if I needed. (…) That it's better to write that I have it than not.” (Interview 2)

> ”He (the father) wanted to try to get benefits. (…) I know he got a lot of benefits from the social insurance agency after I was diagnosed.” (Interview 14)

3.2. Identity and identification: excitement, recognition – and the opposite

Participants described how being diagnosed had led to shifts within themselves and changed self-understanding. One group of participants was clear in that they initially considered their diagnosis to be correct.

> ”Earlier I've recognized myself a lot in it. When I was in school and had a lot of problems.” (Interview 5)

> ”I've sat for countless hours just nerding out and checking: Do I really fit into this? When I was younger, absolutely.” (Interview 12)

Some described initial feelings of excitement about being diagnosed, indicating that they were special or seen as proof of something not being right.

> “It sounded cool. (…) I felt a bit unique.” (Interview 9)

> “I was happy at first because: Yes, there is something wrong with me.” (Interview 8)

Another group of participants described having never identified with being diagnosed.

> ”I never had that feeling where others have said: “Oh, God, how nice, now I have an explanation for why I behave the way I do.” (Interview 4)

> ”I've met many people with Asperger's. (…) [sighs] And I've never recognized myself. (…) How could they put that diagnosis on me?” (Interview 9)

Participants described emotions of being diagnosed, saying that it had made them feel like they were ‘worse than everyone else’ (interview 2). They described having felt frustrated, distressed and disappointed.

> ” I was devastated, ashamed, and didn't feel like it suited me at all.” (Interview 4)

3.3. Formal and informal effects of diagnosis ranging from crucial help to misdirection

Participants said that they had been able to access resources or adjustments in school after having been diagnosed, including changing schools, working in smaller groups, receiving knowledgeable supportive teachers, being admitted despite missing grades and receiving financial aid. Some described these effects as necessary and life changing.

> ”I was able to come to a different school. (…) The teachers seemed to have experience with people with difficulties. (…) I actually believe that saved me.” (Interview 2)

> ”Finally, I was one of the top students the year I graduated. I never would have done that without my ADHD boarding school.” (Interview 13)

> ”She (mother) needed these care allowances (…) for food or for other things that were really, really needed.” (Interview 7)

Participants described experiences of how receiving the diagnosis had improved approaches and understanding from teachers, and that parents had educated themselves because of their diagnosis. These changes improved their well-being and situation in school and at home, making them feel supported and listened to, resulting in higher grades.

> ”Immediately after I got it (the diagnosis), all the teachers understood that I wasn't being like that just to mess with them (…) so they changed their attitude (…) and then everything went well, [laughs] really well.” (Interview 5)

> ”At the habilitation center (…) I had a great supportive contact.” (Interview 8)

Participants also gave examples of resources they had been provided with related to the diagnosis which they said had not made any real difference to them or their situation.

> ”I got this personal assistant; I still wasn't doing any better at school.” (Interview 1)

> ”I got (…) a kind of mat. So, you sit in a chair and there are (…) thorns all around, uh, but I didn't get anything else. (…) Maybe it helped a little bit, as a kind of distraction.” (Interview 6)

Participants testified to changes related to the diagnosis which had been unhelpful. Some participants described how they thought that the diagnosis had drawn attention to the wrong things. Instead of receiving help to, for example, handle difficulties related to parental addiction at home or bullying in school, the diagnosis had resulted in medical interventions or changes in school which had made them feel worse. Some participants described negative experiences with diagnosis-related medication.

> ”I’ve never felt like I’ve functioned any better on (…) ADHD medication. *Did you experience any negative effects from it?* Yes, I did. Anxiety, paranoia, decreased appetite, changed personality. (…) *And did you tell anyone?* Yes, all the time. (…) So, I went through all of them without any improvement. The biggest and best improvement was when I stopped taking it. Strange.” (Interview 9)

> ”They never brought up the bullying. It was just about me losing interest easily, and their first solution to that was medication. (…) So, in the end I had teachers and parents saying: “Yes, but take medication”. But I didn’t want to.” (Interview 13)

Participants reported adverse spatial consequences associated with their diagnosis, including social isolation, and struggles to integrate, as they were separated from classmates, moved to a new school or put in separate rooms, where they had not followed the curriculum, felt lonely and did not feel like they fit in.

> ”I wasn't allowed to learn regular subjects. I wasn't allowed to learn math and Swedish, English, because we were given something else. So, I was locked in a room and kind of had to do this computer … computer stuff.” (Interview 7)

> ”I didn't want to be alone in a different room doing, uh, assignments. I wanted to be in my class.” (Interview 10)

"It was too damn awful, because my friends were at the old school. (…) There were only boys at the school I changed to. I think there were one or two girls, but they were also like boys." (Interview 12)

3.4. Motive and rationale for wanting reassessment

Going deeper into the reasons for seeking reassessment below follows a description of results organized into the four main themes. Being interrelated and not easily separated the first section covers aspects on motive and rationale related to descriptions of who the participants were (Being no different and Having capacity) and the second to what the participants aspired to achieve (Demanding fairness and Removing barriers).

3.4.1. Being no different and having capacity

Participants discussed how they in their current life were able to handle things on their own and lead a normal life, which had made them think about reassessment of the diagnosis. One aspect of realizing that they might not have the diagnosis was related to the discovery of not needing any treatment or support. School results in high school were described by several participants as a turning point, where studies were largely based on individual interests provided in a new context. They described feeling more motivated, able to work and focus on things that interested them based on their own choices, of making friends and no longer experiencing social issues.

"When I started high school, I didn't feel like I … well, I didn't get any special help there, but I managed anyway. Because I made friends and things like that. Who encouraged me and made me want to go to school." (Interview 2)

"One summer holiday when I didn't take (the medication) anymore (…) I felt like: "Oh, I'm kind of the same with the medicine as I am without it." (Interview 5)

"It's going really well. I can handle things on my own and stuff. And so, I've kind of thought: But does it really fit with me? Like, what was it like when I was younger compared to now? What has changed? Or, like, was it incorrect?" (Interview 7)

Different levels of emotional investment behind the initiative to get a reassessment was reported. Some described how it was about taking the opportunity and 'check to see if it is gone or still there' (Interview 1), where the true answer was the goal rather than a certain outcome.

"It'll just be nice to have it confirmed. Whether I actually have ADD or not. I just want to know. Regardless (…) I'll be happy. Because then I'll know for sure." (Interview 7)

Some participants described having considered the risks of getting a reassessment as persons close to them had warned them about losing access to future support. Others described having wanted diagnosis reassessment for years and how they had tried to have it removed, in some cases going back all the way to the time when they had been diagnosed. Being an adult had given them the means to initiate a reassessment.

"After I got my diagnosis, I felt like, "No, this doesn't apply to me." So, I tried to contact CAP. I tried to change schools. I tried to persuade my mother, but without results. (…) Now that I've turned 18, I was able to reach out to Lund university and this experiment and ask for help." (Interview 12)

Some participants stated how they had been opposed by parents or others around them who did not support the idea of reassessing the diagnosis. Participants expressed frustration over feeling that they were seen by people close to them as who they once were rather than who they were today. However, more so they described support from others, for example a parent, a teacher, a school nurse or friends who were saying that they saw no signs of them having a diagnosis and encouraging them to have it reassessed. Participants said that when everyone around you says that it makes sense to question the diagnosis, they had begun to feel even more that they did not fit into it. Participants described how they perceived that they had caught up with themselves as they were living a more normal teenage life, looking back on themselves as young and easily impressionable, but since then having grown, making the diagnosis no longer relevant to them.

"The brain develops over time, and I believe that things like that can change. (…) If you learn to find ways to deal with it, you don't have a problem anymore and then there's no diagnosis, right?" (Interview 5)

"Maybe even I thought that it was right, but now I don't. It doesn't make sense." (Interview 10)

"I want to feel like everyone else. I don't want to feel special." (Interview 3)

Participants described having capacity and influence to initiate a process on their own terms.

"I'm the one who applied here now, eh, and, like, have done it on my own terms. And it's not just *one* person who gets to talk, but several people are discussing. And that feels really safe." (Interview 4)

3.4.2. Demanding fairness and removing barriers

Looking back on experiences of unfair diagnostic processes and difficult life circumstances, participants were motivated to have the diagnosis reassessed for fairness and rectification. Several participants questioned the diagnosis, pointing toward alternative explanations, such as having been depressed, living with parental addiction or being the victim of controlling parents with an agenda.

"I want to get rid of it because I felt they did so much wrong, so it's almost a little bit about (…) revenge." (Interview 8)

"Now being adult, I understand that my diagnoses are probably my mother's scapegoat (…) for everything that happened during my childhood. (…) Why should I carry that label when it's obviously so wrong? It's not fair." (Interview 9)

"I've always felt like this was something my father tried to get through. (…) Now, when I have left my father, I want to get rid of everything I felt wasn't quite right." (Interview 14)

Participants were talking about how seeking reassessment was not only about themselves but also about contributing to change.

"We need to share because it can help others." (Interview 1)

"There are many who feel the same way I do. (…) I thought about this for so many years, but there was never anywhere to turn. Uh. It is so much easier to receive a diagnosis than to get rid of one." (Interview 4)

Participants were saying that a diagnosis needed to be up to date, that it is unnecessary to go around carrying a diagnosis which they could no longer identify with. They questioned that these diagnoses were not checked upon regularly, with reference to how much they thought their behavior had changed since the time of the first assessment.

"Shouldn't they keep it (the diagnosis) up to date?" (…) It was basically almost nine years ago now, not much of that diagnosis is visible today." (Interview 6)

"You want to have the right diagnosis. It feels strange to walk around with a diagnosis that you don't have." (Interview 8)

Participants described how the diagnosis had made them feel like they had lost their life's purpose. Realizing that a diagnosis could prevent them from working towards their dream job had made them lose motivation. The potential effects the diagnosis could have on their

future had decreased their functioning in school, creating a downward spiral. Seeking reassessment of the diagnosis was a potential way to remove systemic hurdles and fulfil dreams of applying for military service, becoming a policeman or taking a driver's license without needing a medical permission.

> "Oh, what does it take to become a police officer? Yeah, no one with ADHD." Well, [sighs] almost everything fell apart, because that's what I wanted. (…) Why should I spend time in school when I can't even become what I want to?" (Interview 1)

While some were quite critical about having received the diagnosis, others were thankful.

> "If I hadn't gotten them (the diagnoses), I might not have even made it … I mean, finished high school, and then I could have completely forgotten about considering joining the military or the police." (Interview 2)

Among the fourteen who were interviewed one participant said they had always and were still identifying themselves with the diagnosis yet wanted to have it removed because it was required by the system to be able to fulfill the dream of becoming a policeman.

> "My ADHD isn't really something I want to get rid of. Because I still want to be able to show that: "Yes, I am a police officer, I have ADHD." (Interview 13)

Participants described that removing the diagnosis could be a way to address limited access to healthcare.

> " I hit the famous wall, uh, felt really bad, and sought care for sleeping issues and anxiety, and then it was immediately: "Yes, but you have ADHD, you need to seek psychiatric care." But (…): "I haven't received any help with ADHD, I feel like these are two separate things". But she still sent me to some psychiatric assessment unit (…) And their assessment was that: 'no, you have nothing to do with psychiatry at all, this is something for the primary care level to handle, you have to go back there'. I went back there, and they said: 'No, but you need to go to psychiatry.' So, [laughs] and I just need help, I'm going … like: "I'm exhausted, I need help for this. Stop talking about ADHD." (Interview 4)

Participants explained that the diagnosis affected their sense of freedom and choice in general terms, making them wish to have it reassessed and hopefully removed. They described the diagnosis as a constant companion which could turn up anywhere, creating a sense of non-transparency. This made them feel trapped, not seen as who they were and set up to fail. They described how they suspected but not always knew that the diagnosis had played a role in a situation – and that it would continue to do so unless it was removed.

> "But why haven't I received a letter?" And then I called the military. (…) And then she said that it's not possible, like, there's something here that means I can't join. And she refused to say what it was." (Interview 7)

> "It follows you like some bloody cancer that plagues your life in every aspect and I could never feel normal. (…) This is just hard. It adds nothing. (…) It takes away my dignity." (Interview 9)

> "In a way I just want it to be normal. (…) Maybe it doesn't affect me now, but I just want to get rid of it." (Interview 14)

4. Summary of findings

The interviews revealed a wide range of experiences and perspectives among participants who sought a reassessment of their ASD and/or ADHD diagnoses. Participants described the tension between access to support and restricted opportunities, and how the diagnosis continued to "travel" across systems long after school, affecting decisions in ways that were often opaque to them. Many described how their diagnoses had been initiated by concerned parents, often with support from teachers or other professionals, in response to behaviours perceived as problematic or abnormal. While some accepted their parents' intentions as well-meaning, others recalled feeling confused, unheard, or even coerced into diagnostic procedures that, in retrospect, felt misaligned with their own experiences.

A common justification for diagnosis was its instrumental value: it provided access to educational, social or financial support. However, participants also reflected on how behaviours that led to diagnosis—such as restlessness, anger, or social withdrawal—could be understood as responses to difficult life circumstances, including trauma or unsafe home environments. The diagnostic process was often described as adult-driven, with little space for the child's voice, and participants questioned whether their experiences were adequately considered in context.

The consequences of receiving a diagnosis were described as both enabling and limiting. Some participants gained access to resources that improved their school situation or well-being. Others described how the diagnosis had misdirected attention away from more pressing issues, led to unwanted interventions (such as medication or school segregation), or contributed to stigmatization. Several described feeling that their diagnosis followed them into adulthood, shaping others' perceptions and limiting opportunities in education, healthcare, employment, and military service.

Participants' identification with their diagnoses varied. Some had never truly identified with the label, others had only initially embraced it, and one still found it meaningful but sought its removal for practical reasons.

The desire for reassessment stemmed from the wish to correct what felt like an inaccurate or unjust categorization, to overcome institutional barriers, personal growth and change, and a hope to gain clarity on current self-understanding. Notably, many emphasized the importance of undergoing reassessment on their own terms—voluntarily, knowingly, and with a greater sense of agency.

5. Discussion

5.1. Diagnostic durability and temporal reach

Our findings highlight the *durability* of neurodevelopmental diagnoses across time and contexts. Several participants described how their ASD and/or ADHD diagnosis/-es continued to "stick," long after they had ceased to be in contact with psychiatry or in need of treatment or support, raising questions about the proportionality between initial benefits and ongoing consequences over time (van Dijk et al., 2016). Previous research has shown that psychiatric diagnoses can affect how young people understand themselves and relate to others. O'Connor et al. (2018) demonstrate that diagnostic labels can threaten self-concept through experiences of stigma and invalidation but also provide validation and belonging. Our participants' reflections on growing out of or moving beyond childhood diagnoses resonate with this ambivalence, as they negotiated between identification with and rejection of diagnostic identities.

The tension between personal development and persistence of diagnostic identity echoes previous qualitative work. Honkasilta et al. (2016) have shown how young people negotiate ADHD as part of their self-concept, oscillating between self-pathologizing and self-liberating understandings of their behaviour. Our participants reveal similar struggles to redefine themselves beyond the diagnostic narratives established in childhood. The label persisted in medical records and institutional registers affecting driver's license applications and military service recruitment. This persistence is not incidental but built into the infrastructures of record keeping and bureaucratic coordination. Turowetz and Maynard (2019) show that diagnostic categories are produced and stabilized through documentation practices that inscribe

clinical judgments into durable institutional artefacts. Once entered into medical records, diagnoses become circulating facts that travel across systems and acquire authority independent of the individual's present condition. Attention to the materialities of care helps explain this persistence: records, letters, and forms mediate how diagnostic labels endure and circulate beyond clinical encounters (Buse et al., 2018). In this sense, the diagnosis had a life of its own, shaping opportunities and exclusions irrespective of present functioning. Such temporal durability complicates narrow understandings of overdiagnosis as a matter of initial misclassification, suggesting the need to consider how diagnoses may outlive their clinical relevance.

The original diagnostic process was often initiated at the request of parents and teachers in response to behaviors experienced as disruptive or difficult. As several participants reflected, the problem was not necessarily that *they* had difficulties with school, but rather that *school* had difficulties with them. This echoes wider sociological concerns of how psychiatric diagnosis can individualize social or relational problems. As O'Connor et al. (2018) highlight, diagnostic labelling often redirects attention from contextual or interpersonal difficulties, such as family instability or trauma, toward individual pathology. Such a shift shapes how both professionals and young people understand the source of distress. From this perspective, the original diagnosis served to manage the needs of others - to explain behavior and secure support - more than to reflect the child's own understanding or experience. Similar dynamics are noted in ASD research focusing on children's perspectives. Karni-Visel et al. (2024) show how diagnostic and treatment encounters often marginalize children's voices, as adults, (professionals and parents), define the problem. Our participants' retrospective accounts can be read as efforts to reclaim interpretive authority over experiences once narrated by others. Today the same individuals navigate workplaces, higher education, the military, and health-care settings. The subject of the diagnosis is thus no longer the child with limited voice but an adult capable of self-advocacy in new institutional arenas. This illustrates how diagnostic categories extend beyond their initial clinical purpose and continue to act as social classifications across life (Jutel, 2009, 2024; Conrad, 2007). As Hacking (2006) has argued, categories "make up people" not only at the point of classification but through the trajectories they enable and foreclose.

5.2. Beyond intended support: unintended effects across domains

The participants' accounts also demonstrate how diagnoses, initially intended to provide access to resources, travel into domains far removed from their original purpose. Several participants explained how their diagnosis closed off professional opportunities or introduced additional requirements. Others described how their diagnosis was invoked in health-care settings attributing unrelated problems to the diagnosis. In everyday life, participants reported feeling labelled and perceived as different, even when they no longer identified with the diagnosis.

These findings resonate with sociological analyses of medicalization, where diagnoses expand their reach by offering medical explanations for behavioural or cognitive difficulties, often replacing structural or pedagogical critiques (Conrad and Bergey, 2014; Singh, 2002). In line with Sims et al. (2021), our participants' stories show how a diagnostic label may simultaneously enable support and impose barriers, becoming a sorting device used by institutions to regulate eligibility, access, and participation. This dual function is precisely what makes near-threshold cases analytically salient: they illustrate both the benefits and unintended burdens of diagnostic expansion (Conrad and Bergey, 2014; Busfield, 2017). From the perspective of overdiagnosis debates, such cases illustrate how the balance between benefit and burden may shift across institutional contexts and life stages, also when initially being perceived as highly validating. Importantly, such effects were visible across some of the most powerful organizing frameworks of society - education, employment, military service, law enforcement, and the family - domains in which participants described their diagnosis as a direct barrier to full participation.

In interpreting these findings, it is important to situate them in the Swedish context. Sweden has anti-discrimination legislation covering disability Diskrimineringslag (2008:567), and automatic exclusion from military service or police education on the basis of ADHD or autism has in recent years been challenged and replaced, at least formally, by requirements for individual assessment. Nevertheless, participants' accounts indicate that diagnoses may still become consequential in specific institutional encounters. The issue, then, is not simply whether a diagnosis formally excludes a person from participation, but how it may become a practical barrier, a source of uncertainty, or something requiring explanation, documentation, or reassessment in relation to future opportunities.

5.3. Reassessment as a response to stickiness and unintended effects

Participants' wish for reassessment can be understood as an effort to regain control over classifications that no longer fit their lives and as a potential response to no longer being proportionate to current circumstances. For some, this was framed in terms of fairness: correcting a perceived misdiagnosis, often linked to life circumstances. Others wished to remove practical barriers create by diagnosis, or moving on from an identity they did not recognize.

The very fact that individuals actively seek, and institutions often require, a formal act of diagnostic removal is telling. It highlights how psychiatric classifications retain weight long after their clinical relevance has faded and raises questions of epistemic authority and responsibility: What does it mean, ethically and clinically, to acknowledge that a diagnosis once helpful has become a source of limitation or harm? As Jutel (2016) argues, diagnosis is not merely a technical decision but a socially and existentially charged moment that rewrites the patient narrative, shifts identity, and dictates the clinical pathway. The act of removal, or diagnostic reversal, can also be seen as a transformative social event. For our participants, a new clinical assessment reconfigured how they were seen by institutions and themselves, marking a transition from being defined by a diagnostic past to reclaiming authorship of their present identity. Who has the power to say that a diagnosis no longer applies to individuals who are living with consequences of a classification made in their name but not necessarily on their terms? To what extent do other institutions accept such reassessments?

Another challenge concerns how reassessment requests are perceived within psychiatry. A common critique is that patients arrive with an "agenda"— answering assessments in ways that maximize the likelihood of removal. Yet, in a broader sense, all patients come to healthcare with an agenda: seeking recognition, relief, or change. In our case, the agenda is inverted: the suffering stems not from the absence of a diagnosis, but from living with one. This "patient who wants to be seen as well" raises a different kind of diagnostic dilemma: one that concerns not only the validity of past classifications, but also the limits of institutional trust, the distribution of epistemic authority and responsibility, and the evolving relationship between identity and diagnosis over time.

6. Methodological reflection

Recruitment based on requests for reassessment without advertising means that our study may add information based on actual needs in the target group. Individuals who proactively request renewed assessment are not intended as a proxy for the ASD/ADHD population. They are informative because they live with, and actively contest, the consequences of diagnostic classification at the widened edges of these categories. Their requests for reassessment make visible how diagnostic thresholds are not settled once and for all at the moment of diagnosis, but may become newly consequential as people move across life stages, institutional settings, and social roles. In this sense, reassessment requests can be read as empirical signals within broader debates on

diagnostic expansion and overdiagnosis.

7. Conclusion

In relation to debates about diagnostic expansion and overdiagnosis, we argue that reassessment cases illuminate critical dynamics in the governance of diagnosis today. They show how diagnoses such as ASD and ADHD extend far beyond the clinic, acquire unintended effects across domains, and generate demands not only for classification but also for declassification. In this sense, the group we studied may not be statistically representative, but they exemplify a broader sociological phenomenon: the growing need to negotiate not only the expansion but also the proportionality and temporal durability of psychiatric diagnoses in contemporary society.

Ethics approval

The research protocol (reg. number 2022-06213-01) was approved by the Swedish Ethical Review Authority.

CRediT authorship contribution statement

Rose-Marie Lindkvist: Conceptualization, Data curation, Formal analysis, Investigation, Methodology, Software, Validation, Visualization, Writing – original draft, Writing – review & editing. **Katalin Niklasson:** Conceptualization, Data curation, Formal analysis, Methodology, Project administration, Software, Validation, Visualization, Writing – original draft, Writing – review & editing. **Allan Lidström:** Conceptualization, Data curation, Formal analysis, Investigation, Methodology, Software, Validation, Visualization, Writing – original draft, Writing – review & editing. **Michelle Nilsson:** Conceptualization, Methodology, Visualization, Writing – review & editing. **Sophia Eberhard:** Conceptualization, Methodology, Project administration, Supervision, Visualization, Writing – review & editing. **Peik Gustafsson:** Conceptualization, Methodology, Supervision, Visualization, Writing – review & editing. **Sebastian Lundström:** Conceptualization, Funding acquisition, Methodology, Project administration, Resources, Supervision, Visualization, Writing – review & editing. **Maria Råstam:** Conceptualization, Methodology, Supervision, Visualization, Writing – review & editing. **Lena Eriksson:** Conceptualization, Data curation, Formal analysis, Investigation, Methodology, Software, Supervision, Validation, Visualization, Writing – original draft, Writing – review & editing.

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Appendix A. Supplementary data

Supplementary data to this article can be found online at https://doi.org/10.1016/j.socscimed.2026.119549.

Data availability

The data that has been used is confidential.

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